PC-02 Pediatric Bradycardia With Poor Perfusion

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 26, 2026

Scope: Infant or child after the newly born period with bradycardia and hypotension, altered mental status, shock, weak pulses, or other poor perfusion. Correct oxygenation and ventilation before medication.

Field flow

1. Assess	Bradycardia with poor perfusion: hypotension, altered mental status, shock, weak pulses, or impending arrest.
2. Correct cause	Airway, oxygenation, ventilation, obstruction, hypoxia, apnea, glucose, temperature, acidosis, toxins, and other reversible causes.
3. HR <60?	If HR remains below 60/min with poor perfusion despite effective oxygenation/ventilation, begin CPR immediately, even with a pulse.
4. IV/IO + epi	Obtain rapid access without delaying CPR. Epinephrine 0.01 mg/kg IV/IO every 3-5 min while compromise persists.
5. Specific cause	Atropine only for increased vagal tone or primary AV block; consider pacing for selected refractory conduction disease.

Provider-level actions

Level	Approved actions
EMT	Airway support, effective oxygenation/ventilation, glucose assessment when authorized, and CPR for persistent HR below 60 with poor perfusion.
AEMT	All EMT care plus IV/IO, epinephrine, glucose assessment, crystalloid only when hypovolemia is suspected, and authorized advanced airway support.
Paramedic	All prior care plus monitoring, atropine for increased vagal tone or primary AV block, and selected pacing for refractory conduction disease.

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SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

- Atropine is not a treatment for hypoxic bradycardia.
- Do not delay oxygenation, ventilation, or CPR for vascular access or medication.
- If pulseless, move immediately to PC-01/PC-04; after sustained ROSC, use PC-08.
- Obtain 12-lead only when it will not delay resuscitation.

Medication reference

Medication	Approved dose / safeguard
Epinephrine 0.1 mg/mL	0.01 mg/kg IV/IO; max single dose 1 mg; repeat every 3-5 min.
Atropine	0.02 mg/kg IV/IO only for increased vagal tone or primary AV block; min 0.1 mg; max single dose 0.5 mg child / 1 mg adolescent; may repeat once.
Normal saline	10-20 mL/kg IV/IO only for suspected hypovolemia; reassess after each bolus.

Linked pathways

AR-05 Pediatric Airway; PC-01 Asystole / PEA; PC-04 Pulseless Arrest; PC-08 Post-Resuscitation; AO-02 Newly Born.

Source control

- This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 26, 2026.
- Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.